



Young People (16-18 years) Admitted to Mimidi Park (Mental Health) Guideline

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

- Part 2, Division 1 of the MHA 14 defines the term child as a person who is under the age of 18.
- Part 18, Division 3, sections 299-304 pertain to “children who have mental illness”.
- Young people aged 16-18 years old, with an acute episode of serious mental illness, requiring inpatient hospital treatment may only be admitted to Mimidi Park Mental Health Inpatient Unit in exceptional circumstances, if there are no places in other age appropriate facilities.
- Admissions aged 16-18 years require direct approval from the RKPG Mental Health Service Clinical Director (or delegate).
- The service clinical director (or) delegate must give to the child’s parent or guardian a written report setting out:
 - i) The reasons why the person in charge is satisfied The mental health service can provide the child with treatment, care and support that is appropriate having regard to the child’s age, maturity, gender, culture and spiritual beliefs; and The treatment, care and support can be provided to the child in a part of the mental health service that is separate from any part of the mental health service in which adults are provided with treatment and care if, having regard to the child’s age and maturity, it would be appropriate to do so.
 - ii) The measures that the mental health service will take to ensure that, while the child is admitted as an inpatient, the child is protected and the child’s individual needs in relation to treatment and care are met; and,
 - iii) File a copy of the report and give another copy to the Chief Psychiatrist (Section 303, MHA 2014).

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Oct 2010	Revisions	May 2018	National Standards for Mental Health 1.9; 2.1; 2.6; 2.11; 2.12; 2.13; 4.1; 4.5; 6.2; 6.5; 6.16; 7.2; 7.5; 7.9; 7.10; 7.11; 7.12; 8.10; 10.1.10; 10.2.1; 10.3.2; 10.3.3; 10.3.4; 10.3.5; 10.3.6; 10.4.3; 10.4.4; 10.4.8; 10.5.1; 10.5.2; 10.5.5; 10.5.9.
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- The Nurse Unit Manager (NUM) (or delegate) must be informed of a young person aged 16-18 years being admitted to Mimidi Park.
- The earliest opportunity for the transfer of the young person to State Youth Inpatient facilities will be actively sought by the Consultant Psychiatrist.
- In the event that a young person is admitted to Mimidi Park, special care arrangements will be put in place to manage their vulnerability and ensure their safety. This will include:
 - * Any young person will have continuous visual special observation (1:1) for the duration of their stay.
 - * When allocating staff for specialising duties clinical history (including trauma issues) and gender of the patient will be taken into account
- Careful consideration needs to be given to the emotional and social maturity of the young person in their capacity to give informed consent and manage the adult environment.
- The treating team for a young person in Mimidi Park will undertake a daily review and consultation with the Child and Adolescent Mental Health Service (CAMHS) team.
- The young person will have their risk assessed and managed in a developmentally appropriate fashion and will be invited to be fully involved in all aspects of care planning.
- The parent(s) or person with parental responsibility should be fully involved in the treatment planning where the young person consents, including the interview process as set out by Section 301 of the MHA 2014.
- If clinically indicated a status of mature minor may be granted. A person under 18 years who has sufficient emotional and intellectual capacity to fully comprehend the nature, consequences and risks of a proposed action (for example, a treatment decision or a decision to release health information), irrespective of whether a parent/ substitute decision maker consents to it.
- When assessing the capacity of children under 18 for informed consent, clinicians should refer to the document "Assessing Capacity to Consent in Minors: A Clinician's Guideline".
- The best interests of the child are a primary consideration; however this can be overridden in circumstances such as where the child is a risk to another person, a risk which cannot be ignored.
- A child who is assessed to be a mature minor in relation to a particular treatment decision may either give or refuse consent to treatment. Please refer to Section 302 of the MHA 2014.
- The multidisciplinary team will ensure that any therapeutic activities or group work undertaken with young person is appropriate to the individual's emotional and social maturity.

Compliance monitoring

Compliance with this guideline will be monitored by any clinical incidents via Datix.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[Mental Health Commission. Australian Government. \(2010\). National Standards for Mental Health Services. Commonwealth of Australia.](#)

[WA Health Consent to Treatment Policy](#)

[Mental Health Act 2014](#)

National Standards for Mental Health Services

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